

QUESTIONS & ANSWERS · OCTOBER 2026

Why we don't share Dr Heal Ad Account 1

The new marketer's Meta ad conversions are very low. Giving them our ad account or Meta API access would not fix that. It would hand over our full playbook and could pull down our own results. Here are the answers, and what we offer instead.



Builds on **Report 1 · Call Attribution**: only calls from the new marketer's own Meta ads count.



Dr Heal Ad Account 1

HOSPITAL-OWNED · NOT SHARED

Campaigns	Private
Audiences	Private
Frequency settings	Private
Budgets and amounts	Private
Metrics and reports	Private
Meta API access	Private

Q1

“Why not give the new marketer access to our ad account or the Meta API?”

ANSWER

Access to Ad Account 1, or to the Meta API, shows **everything**: every campaign, audience, frequency setting, budget and metric, from A to Z.

It is the full result of our own testing and spend. With access, anyone can copy the whole setup in a day and present it as their own work.

**Access = our complete playbook.
So the answer is no.**



What anyone with access would see

Campaign structure

Which campaigns and ad sets run, and how they are built

Audiences and targeting

Who we reach: areas, ages, interests, custom audiences

Frequency

How often each patient sees our ads before calling

Budgets and amounts

What we spend, where, and how it is split

Metrics

Cost per call, conversions, what works and what does not

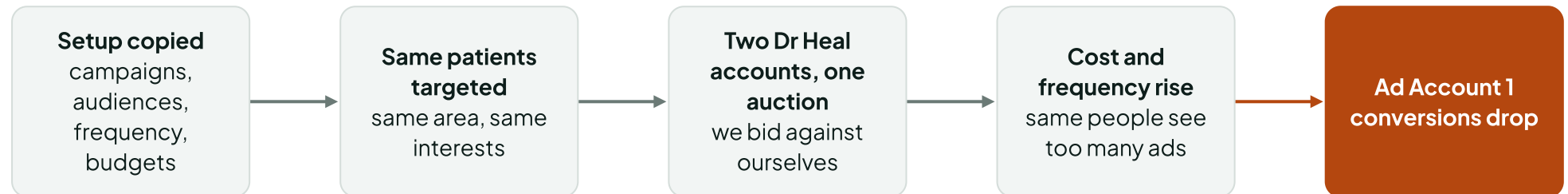
Creatives and copy

Every ad, video and message we have tested

Q2

“What actually goes wrong if they copy our setup?”

Answer: our own ads start competing against a copy of themselves, and Ad Account 1 conversions go down.



We pay twice

Two accounts chasing the same patient in HSR Layout push up our cost per call. The hospital pays both bills.

Ad fatigue sets in

Frequency is a setting we have balanced with care. A copy doubles it, and patients start ignoring Dr Heal ads.

The copy proves nothing

Any calls a copy brings are our strategy's results, not the new marketer's. We still would not know their real skill.

Q3 “Their conversions are low. Wouldn't access help them improve?”

Answer: low conversions are a reason to keep access closed, not to open it. Access gives them our answers. It does not build their skill.

WHAT ACCESS WOULD GIVE THEM

- ✗ Our audiences, ready to reuse
- ✗ Our winning campaigns, ready to copy
- ✗ Our results, which they could show as theirs

WHAT WE HIRED THEM FOR

- ✓ New ideas and new audiences
- ✓ New calls, on top of the calls we already get
- ✓ Results we can measure in their own account

01

The answer is not the skill

Seeing our setup teaches copying. A marketer must learn to find patients on their own.

02

We would pay twice for our own work

If they need our account to perform, we are paying a new salary for results we already have.

03

It breaks the counting rule

Report 1: only calls from their own Meta ads count. Mixed accounts make that impossible to see.

Q4

“So what will the new marketer get from us?”

Answer: everything they need to build their own campaigns. Nothing that reveals ours.

✓ We share

Brand kit

Logo, colours and approved photos

Hospital information

Treatments, doctors and timings to use in ads

Approved messages and offers

What the hospital is happy to promise

Their own test budget

A fixed amount, in their own ad account

Weekly call-quality feedback

From our front desk, on calls their ads bring

🔒 Stays private

Dr Heal Ad Account 1

No role of any kind: admin, advertiser or analyst

Meta API access

No tokens, no app connections, no tools that read our account

Audiences

Saved, custom and lookalike audiences are not shared

Campaign settings

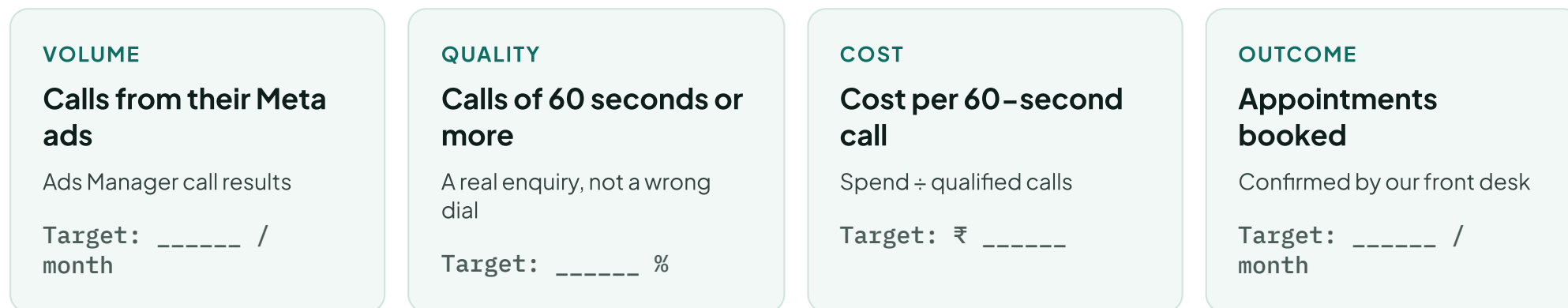
Structure, frequency, budgets and bidding

Reports and metrics

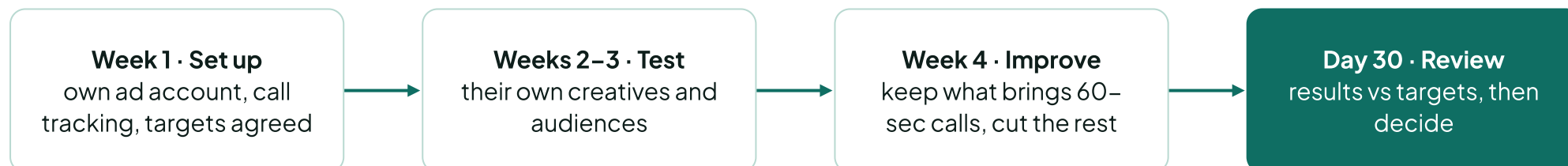
No screenshots or exports of Ad Account 1 results

Q5 “How will we judge whether the new marketer is working?”

Answer: four numbers from their own Meta ad account, reviewed after 30 days against targets we agree up front.



THE FIRST 30 DAYS



Not counted: calls on ...8000 or ...8086, walk-ins, or any result from Dr Heal Ad Account 1. See Report 1.

QUICK ANSWERS

If they ask, this is what we say

“Can I get view-only access?”

No. Even view-only shows our audiences, budgets, frequency and results: the same playbook.

“Can you share the Meta API access?”

No. API access can read the whole account and copy it in minutes. It stays with the hospital.

“Why aren't calls on ...8086 counted for me?”

They come from Google searches driven by our ₹2 lakh brand ads. Only calls from your Meta ads count. See Report 1.

“How do I improve without seeing what works?”

That is the job we hired you for. Test your own audiences and creatives. We give weekly call-quality feedback.

“What if my results stay low?”

We review at Day 30 against the targets we agreed, and decide on the numbers.



Our ad account belongs to the hospital. New marketers grow calls with their own work, in their own account.